

Public Health Emergency Operations Center

"COUSP DRC"

MVE-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°066/MVB_19/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (47)	• Haut-Uélé (4/13): Wamba, Boma Mangbetu, Pawa and Isiro • Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. • South Kivu (1/34): Miti-Murhesa • Tshopo (3/23): Makiso-Kisangani, Lubunga and Mangobo
Reporting date	July 19, 2026
Publication date	July 20, 2026

KEY INDICATORS — MULTICURRENT AS OF JULY 19, 2026

CONFIRMED CASES — 5 PROVINCES 2,423 +79 new cases (Ituri 70 · N-Kivu 9)	CONFIRMED DEATHS · LEATHITS 967 · (39.9%) +37 deaths (Ituri 35 · North Kivu 2)	SUSPECTED CASES OF THE DAY 252 Ituri 207 · N-Kivu 41 · Haut-Uele 4
PATIENTS IN ISOLATION 734 Ituri 553 · North Kivu 181	CURED — CUMULATIVE 469 +3 healed (Ituri 3)	NATIONAL CONTACT TRACKING 81.1% 8531/ 10519 views · Ituri 81.3% · N-Kivu 83.8% · Haut-Uele 52.3%

1. Epidemiological Highlights

- **Epidemiological situation** : A new affected health zone has been reported in Ituri province: the **Adja health zone**. The total number of affected health zones now stands at 47 out of 140 across the five provinces, of which 46 remain in active transmission.
- **Coordination** : Joint cross-border technical evaluation of the establishment of the Kasenyi Mobile Laboratory by the DRC and Uganda.
- **Case trends** : In the last 24 hours, 79 new confirmed cases, 37 confirmed deaths and 3 recoveries were recorded:
 - o **Ituri**: 70 new confirmed cases were reported, mainly in Bunia (18), Nizi (16), Adja (10), Rwampara (10), Bambu (4), Mangala (3), Mongbwalu (3), Nia-Nia (2), Ariwara (1), Lita (1), Logo (1) and Tchomia (1). The province recorded 35 deaths, 65.7% of which were community deaths and 12 of which occurred in Ebola Treatment Centers (ETCs).
 - o **North Kivu**: 9 new confirmed cases were reported (Katwa 6, Butembo 1, Musienene 1 and Oicha 1) as well as 2 deaths, including 1 community death and 1 at the Katwa treatment center. The provincial total reaches 239 confirmed cases and 141 deaths, representing a case fatality rate of 59.0%.
 - o **Haut-Uélé, South Kivu and Tshopo** : No new confirmed cases or deaths have been reported. The totals remain at 16 confirmed cases and 10 confirmed deaths in Haut-Uélé, 5 cases, 4 deaths and 1 recovery in Tshopo, and 3 cases and 1 death in South Kivu, where the Miti-Murhesa health zone has gone 53 days without a new confirmed case.

- **Analysis:** Ituri remains the epicenter of the epidemic, accounting for 89.1% of cumulative cases and 88.6% of new daily confirmations. Contact tracing performance remains insufficient and requires strengthened surveillance activities.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) epidemic caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 47 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response interventions, and limit the spread of the epidemic.

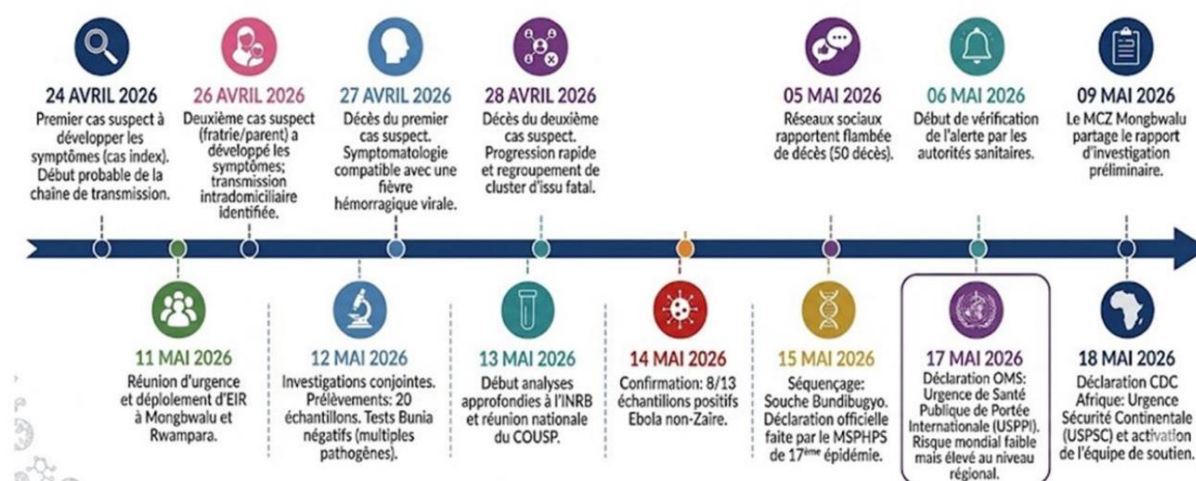


Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Fatality, Affected Health Zones, New Cases	(24 h)
Ituri	2,160	811	37.5% 28 / 36 (77.8%)	70
North Kivu	239	141	59.0% 11/34 (32.4%)	9
South Kivu	3	1	33.3% 1/34 (2.9%)	
Haut-Uélé	16	10	62.5% 4/13 (30.8%)	
Tshopo	5	4	80.0% 3/23 (13.0%)	0
Total	2,423	967	39.9% 47 out of 140	79

Ituri province remains the epicenter of the 17th Ebola virus disease (EVD) outbreak, accounting for 89.1% of confirmed cases (n = 2,160) and 83.9% of deaths (n = 811), representing a case fatality rate of 37.5%. The health zones of Bunia (639 cases), Rwampara (450), Mongbwalu (363), Nizi (214), and Nyankunde (99) remain the most affected, accounting for approximately 81.7% of reported cases at the provincial level and 72.8% at the national level.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	2,160	811	37.5%	70	23	12	35
Bunia	639	211	33.0%	18	8	1	9
Rwampara	450	133	29.6%	10	4	5	9
Mongbwalu	363	203	55.9%	3	0	4	4
Nizi	214	93	43.5%	16	5	1	6
Nyankunde	99	28	28.3%				
Lita	78	27	34.6%	1	0	0	0
Mangala	61	38	62.3%	3	3	0	3
Nia-Nia	51	15	29.4%	2	0	0	0
Bamboo	40	14	35.0%	4	1	1	2
Komanda	27	10	37.0%				
Tchomia	23	6	26.1%	1	1	0	1
Unidentified	17	0	0.0%				
Fataki	11	7	63.6%				
Kilo	11	3	27.3%				
Adja	10	1	10.0%	10	1	0	1
Mandima	9	4	44.4%				
Mambasa	8	6	75.0%				
Logo	8	1	12.5%	1	0	0	0
Ariwara	7	1	14.3%	1	0	0	0
Damascus	7	0	0.0%				
Aungba	6	2	33.3%				
Drodro	5	3	60.0%				
Rimba	5	0	0.0%				
Aru	4	1	25.0%				
Kambala	2	2	100.0%				
Lolwa	2	1	50.0%				
Boga	1	1	100.0%				
Gety	1	0	0.0%				
Mahagi	1	0	0.0%				
North Kivu	239	141	59.0%	9	1	1	2
Katwa	104	72	69.2%	6	0	1	1
Butembo	58	30	51.7%	1	1	0	1
Blessed	37	25	67.6%				
Musienene	19	5	26.3%	1	0	0	0
Kyondo	9	4	44.4%				
Kalunguta	2	1	50.0%				
Masereka	3	0	0.0%				
Oicha	4	3	75.0%	1	0	0	0
Goma	1	0	0.0%				
Vuhovi	1	1	100.0%				
Mabalako	1	0	0.0%				
South Kivu	3	1	33.3%	—			—

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Miti-Murhesa	3	1	33.3%				
Haut-Uélé	16	10	62.5%	—			—
Wamba	6	3	50.0%				
Pawa	3	3	100.0%				
Boma Mangbetu	4	2	50.0%				
Isiro	3	2	66.7%				
Tshopo	5	4	80.0%	—			—
Makiso-Kisangani	3	3	100.0%				
Mangobo	1	1	100.0%				
Lubunga	1	0	0.0%				
Total	2,423	967	39.9%	79	24	13	37

2.3 Mapping of confirmed cases

The spatial distribution confirms a transmission mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwalu, Nizi and Nyankunde, with an extension towards North Kivu (Katwa, Butembo, Beni), Haut-Uélé (Wamba) and Tshopo.

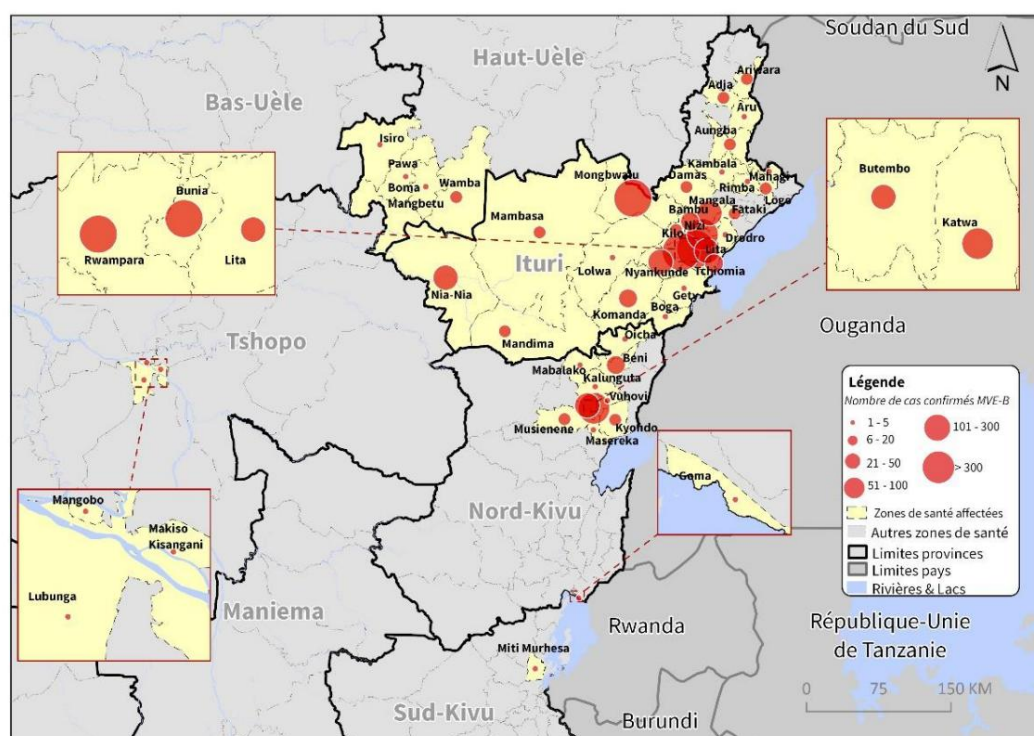
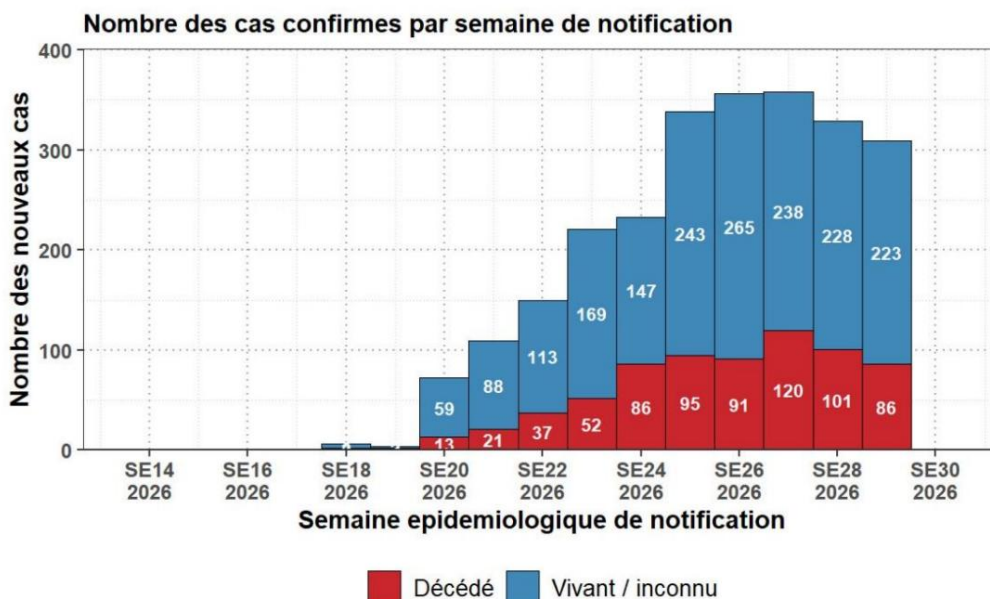


FIGURE 2 — HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES (AS OF 19/07/2026)

The recent emergence of new outbreaks in Haut-Uélé confirms a progressive expansion of the affected areas, highlighting the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

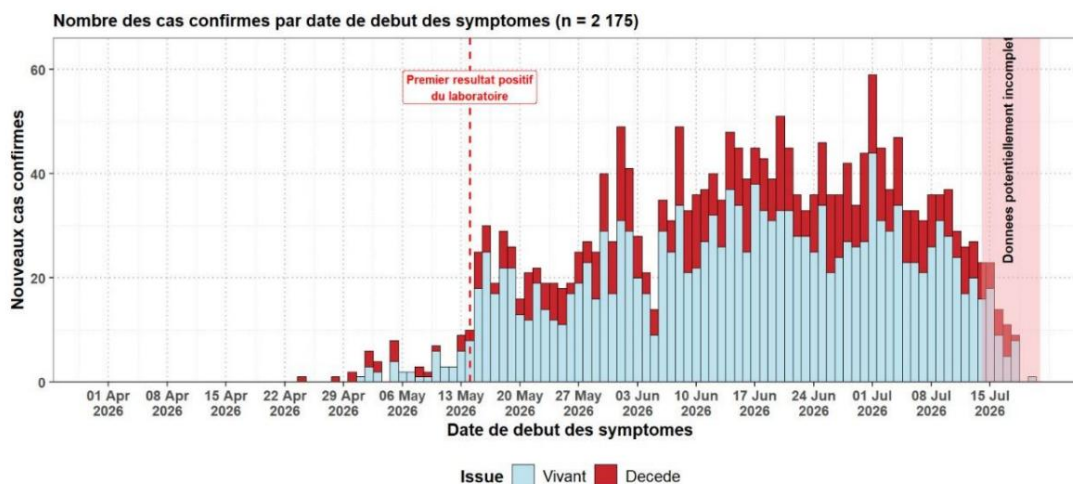
2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained community transmission, with a continuous increase in confirmed cases during epidemiological weeks S25 to S29, each recording more than 300 cases.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses : cas confirmés avec statut vital et date de notification renseignés, n = 2 485.

FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses : cas confirmés avec statut vital et date de début de symptômes renseignés, n = 2 175.

FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults** (18–50 years). Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

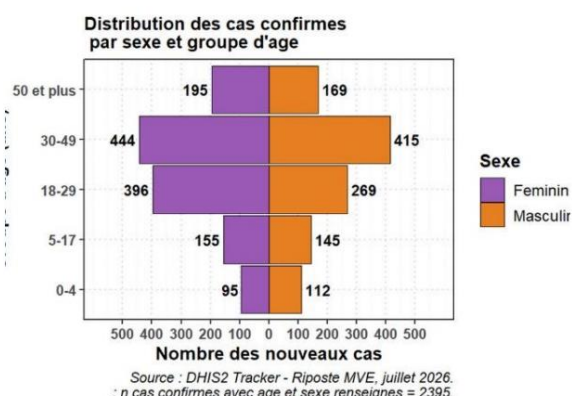
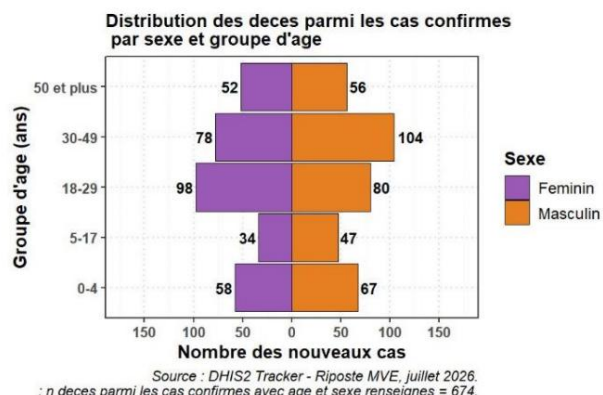


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG THE CONFIRMED CASES)



The distribution of cases by sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS AS OF 19/07/2026

Indicator	Ituri North Kivu		Haut-Uélé	Tshopo	Total
Report (D-1)	26	52	32	ND	110
New alerts received	417	572	4	ND	993
Total alerts for the day	443	624	36	ND	1103
Alerts investigated	335	572	4	ND	911
Investigation rate (24 h)	75.6%	91.7%	11.1%	ND	82.6%
Alerts confirmed — live	154	19	1	ND	174
Alerts confirmed — deceased (comm.)	53	22	3	ND	78
Total suspected cases for today	207	41	4	ND	252

In total, 1103 alerts (993 of which were new) were received, with 911 investigated within 24 hours (82.6%) and 252 confirmed (174 active cases and 78 community deaths). This improvement reflects a reduction in investigation delays and the consolidation of data from the affected provinces.

3.2 Laboratory: tests and positivity

ŷ **Ituri** : 167 samples documented in the collection network, including 70 positive (47 live and 23 dead), i.e. a apparent positivity of 41.9%.

ŷ **North Kivu** : 105 samples received and analyzed, of which 9 were positive (8 living and 1 deceased), representing a positivity rate of 8.6%.

ŷ **Haut-Uele** : 4 samples taken are currently being analyzed.

- Joint technical evaluation (DRC & Uganda) of the Kasenyi Mobile Laboratory, deployed by the Ugandan side.

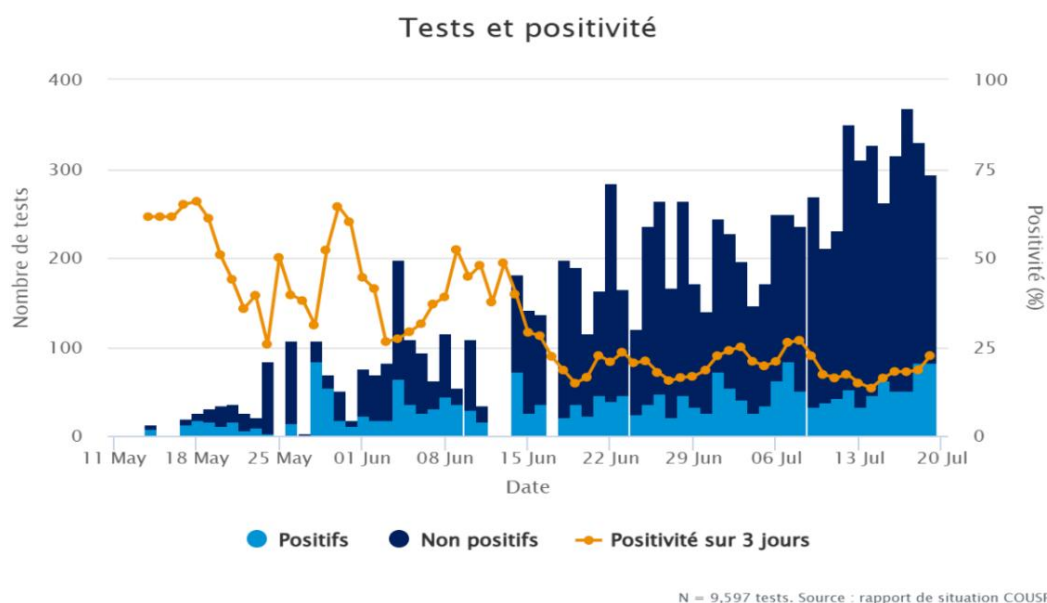


FIGURE 7 — TESTS AND POSITIVITY (COUSP SITUATION REPORT)

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	7581	6167	81.3%
North Kivu	2628	2202	83.8%
Tshopo	ND	ND	ND
Haut-Uélé	310	162	52.3%
South Kivu (end of monitoring)	0	0	N / A
Total	10519	8531	81.1%

Contact tracing remains insufficient at the national level (81.1%), below the operational threshold of 95%. Ituri only managed to follow up with 6,167 out of 7,581 contacts (81.3%). North Kivu maintains a high performance with 2,202 out of 2,628 contacts seen (83.8%). Haut-Uele achieved a low follow-up rate of 52.3%. Follow-up is now complete in South Kivu.

3.4 Entry points and control points (PoE/PoC)

As part of capacity building for the response, the PoE/PoC pillar received an ambulance provided by the SGI; The PoE/PoC pillar benefited from the provision of communication tools by FHI360 to strengthen response activities.

3.4.1 — Crossing point alerts (24 h)

Ituri — 8 alerts notified

Eight (8) alerts were notified to the PoE/PoC in Ituri, including seven live alerts and one intercepted body. Of these, six (6) live alerts were validated and all were referred. Resistance/refusal to transfer was reported, notably to the Mudzibala PoC.

North Kivu — 0 alerts

No alerts have been notified to the various points of entry (PoE) and points of control (PoC).

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC), AS OF 19/07/2026

Indicators	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (15/15)	100% (75/75)
Number of people who have switched to PoE/PoC	94,657	67,652	162,309
% of travelers screened	93.0%	97.12%	94.7%
% of travelers who washed their hands	92.0%	97.12%	94.6%
% of travelers made aware	98.8%	97.12%	98.1%
Number of alerts notified	8	0	8
Number of validated live alerts	6	0	6
Number of bodies recovered today	1	0	1
Number of problematic contacts intercepted today	0	0	0
Percentage of validated live alerts referred to the CT/CTE	100.0%	N / A	100.0%
% of the lifeless bodies swabed	N / A	N / A	0%
Number of positive results of referred suspected cases	0	0	0

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI), AS OF 19/07/2026

Indicator	Ituri	North Kivu	Haut-Uele	Tshopo	Ensemble
Number of beds	798	141	ND	ND	939
Patients in bed (Day -1)	543	177	ND	ND	720
Total admissions (24 hours)	70	38	ND	ND	108
Total admissions	613	215	ND	ND	828
deceased	17	1	ND	ND	18
No case	30	33	ND	ND	63
Healed	3	0	ND	ND	3
Escapees	10	0	ND	ND	10
Transferred to the HGR	0	0	ND	ND	0
Total outflows (24 h)	60	34	ND	ND	94
Patients in isolation (end of day),	553	181	ND	ND	734
including confirmed cases	275	20	ND	ND	295
including suspects	278	161	ND	ND	439
Occupancy rate	69.3%	128.4%	ND	ND	78.2%

In total, 734 patients were in isolation, including 295 confirmed cases and 439 suspected cases, for an overall occupancy rate of 78.2% (734 patients out of 939 beds), with 828 admissions and 94 discharges (3 recovered, 18 deceased, 63 non-cases, and 10 who escaped). Ituri accounts for the majority of the burden with 539 patients in isolation (275 confirmed, 278 suspected) out of 798 beds, representing an occupancy rate of 69.3%. In North Kivu, 181 patients are in isolation (20 confirmed, 161 suspected) for an occupancy rate of 128.4% (141 beds). The provinces of Tshopo and Haut-Uele did not make data available at the time of compilation.

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY INDICATORS SMSPS, JULY 19, 2026

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	6 (33.3%)	0
Confirmed cases are being monitored.	106 (58.6%)	3 (100%)
New suspected cases	25 (59.5%)	4 (100%)
Suspected cases are being monitored	87 (56.9%)	15 (100%)
Laboratory results announced (including positive ones)	33	9
Children separated — nursery / community	5	3
CTE support staff	0	0
Frontline Personnel (PPL)	0	0
Members of households and communities	239	2
EDS supported (SMSPS)	15	0

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVD (CUMULATIVE AS OF JULY 19, 2026, ITURI)

Health Zone	Confirmed cases	Healed	In hospitalization	Death
Bunia	35	18	3	14
Rwampara	32	24	1	7
Mongbwalu	16	6	2	8
Nyankunde	14	11	1	2
Nia-Nia	10	3	7	0
Lita	4	1	2	1
Nizi	3	0	0	3
Mangala	3	1	1	1
Bamboo	2	2	0	0
Kilo	2	1	1	0
Total — case fatality rate 29.8%	121	67	18	36

PPL = healthcare personnel and frontline providers.

7 Pillar-Based Response Activities

COORDINATION Meeting held to coordinate monitoring of response activities in the 5 affected provinces. Joint cross-border technical evaluation of the establishment of the Kasenyi Mobile Laboratory by the DRC and Uganda.	PCI / WASH ÿ Ituri: 8 rings opened around the cases, 23/76 households or places of passage decontaminated within 48 hours (30%), 4 ESS and 4 means of transport decontaminated, 18 ESS monitored/accompanied and 8 PPL trained.
Dignified and secure burials (DSM) ÿ Ituri: Continuation of EDS and swabs, with community resistance reported in Komanda, Mambasa, Mandima and Mangala; continuation of construction work on the decontamination base in Bunia. ÿ North Kivu: 20 death alerts, 20 bodies swabed and secured, 24 EDS carried out and 10 reports of the day.	HOLISTIC CARE (HCC) ÿ Ituri: 3 new recoveries, 70 admissions, 553 patients in isolation, 17 deaths among discharged patients (12 confirmed), 994 hot meals served, and capacity building for 31 psychologists. Backfilling completed and work to expand the CTE in the Lita health zone begins. ÿ North Kivu: 38 admissions, 34 discharges, 181 patients hospitalized, including 20 confirmed and 161 suspected; continued monitoring of care in CTE/CT.
COMMUNICATION (RCCE / CREC) ÿ Ituri: 3,913 people reached during home visits, 875 during educational talks, 415 engaged via community dialogues and 18,870 during mass awareness campaigns; 35 community alerts and 5 feedbacks clarified. Briefing for 50 community leaders including 9 women in the Lita health zone with the support of WHO, ACDC and DPS Ituri. ÿ North Kivu: 23,510 people reached by door-to-door, 5,584 in public places, 178 community alerts, community incident reported in Tuungane requiring security/mitigation intervention.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ÿ Ituri: Monitoring mission in the Nizi Health Zone: Of the six shortlisted healthcare facilities, only two have a functional triage system at the entrance, a separate circuit for managing suspected cases, and a sufficient stock of medications. The cumulative PPL The number of affected cases is 121, with 65 recoveries, 18 hospitalized and 36 deaths.
RESEARCH, INNOVATION & VACCINATION ÿ Ituri: Reporting of research indicators with 10 approved protocols, 3/10 studies completed, 2 reports produced, 1 publication, and continuation of anthropological research in Mongbwalu. Preparation for the KAP field visit to the health zones of Rwampara, Bunia, Bambumines, Kilo, Lita, and Gethy.	LOGISTICS ÿ Ituri: Report on the estimation session PCI, PEC and EDS input requirements, planning of logistical support to the pillars and infrastructure monitoring. ÿ North Kivu : Delivery of supplies to the Health Zone of Musienene and Finalization of the packaging of UNICEF inputs for the health zones of Katwa, Butembo, Musienene and Beni.

PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH)

Continued awareness-raising and strengthening of mechanisms for preventing sexual exploitation and abuse. Activities must be systematically documented in the affected provinces, particularly during the deployment of new response actors.

8 Main challenges, impacts and actions required

Challenge No.		Impact	Actions required
1	Reluctance to participate in post-mortem sampling (EDS/swabs), community resistance, and insecurity targeting EDS teams (bridge attack) Muchanga, paralysis of activities in Nyankunde)	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of the military governor of Ituri)	Interruption of response activities, including dignified and safe burials	Regularize payments to service providers; secure dedicated funding
3	Insufficient capacity of CTE/CT: occupancy of 128.4% in North Kivu (bed capacity: 141) and 69.3% in Ituri (553 patients for 798 beds);	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTES, deploy temporary structures, and systematically report bed capacity.
4	Low performance of contact tracing in Haut-Uele (52.3%), in Ituri 81.3%; North Kivu 83.8%	Uninterrupted transmission chains	Train, deploy and motivate community liaison officers (RECOs); strengthen supervision; investigate the observed drop in Ituri
5	Ambulance shortages and pre-positioning at points of control	Delays in transfer and isolation	Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
6	Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (175/1170, i.e. 15%)	pupil infections Nosocomial risks (121 cases of PPL including 36 deaths)	Urgent supply, strengthening of IPC training
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread	Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of laboratories proximity (Butembo, Mambasa and Logo)	Long confirmation delays, transmission not detected	Accelerate the deployment of decentralized diagnostic capabilities
10	Financial shortcomings of the resources	Limitation of the implementation of the pillars	Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)	Restriction of operations	Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality	Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities



Launch of training on the Ebola virus disease response in the province of Haut-Uele, from July 19 to 20, 2026, by the national, provincial and WHO COUSP.



Briefing of community leaders in the Lita health zone



Finalization of backfilling and start of CTE extension work in the Lita health zone.



Joint cross-border technical evaluation of the establishment of the Kasenyi Mobile Laboratory by the DRC and Uganda.



Briefing of 15 laboratory technicians from Logo, Rwampara, and Nizi on the proper use of essential laboratory equipment, wearing of personal protective equipment, and donation of 2 generators for Nyankunde and NiaNia with support from the US CDC

**POUR TOUTE
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Avec l'appui technique de l'Organisation Mondiale de la Santé (OMS),
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